1. Sepsis and fluids

- = **ProCESS** / **ARISE** / **ProMISe** 2014–15: EGDT no benefit
- ✔ **ANDROMEDA-SHOCK** 2019: CRT-guided ≥ lactate-guided
- = **CLOVERS** 2023 / **CLASSIC** 2022: restrictive = liberal fluid
- ✔ **SMART** 2018: balanced ↓ MAKE30; = **BaSICS**, **PLUS**
- ✖ **6S**, **CHEST**: starch → AKI, RRT, death
- = **ALBIOS** 2014, **SAFE** 2004: albumin = saline (✖ TBI)

Key point: EGDT dead; balanced crystalloids; avoid starch.

2. Vasopressors and steroids

- = **VASST** 2008: vasopressin = NA (possible benefit in less severe shock)
- ✔ **ADRENAL** 2018: faster shock reversal; ✔ **APROCCHSS** 2018: ↓ mortality
- = **SEPSISPAM** 2014: MAP 80–85 not better; **65 trial** 2020
- ✔ **ATHOS-3**: angiotensin II raises MAP
- ✖ **LOVIT** 2022: vitamin C harm; = VITAMINS, VICTAS
- = **STRESS-L** 2023: landiolol no benefit

Key point: MAP 65, NA first, add vasopressin and hydrocortisone.

3. Antibiotics

- = **STOP-IT** 2015: 4 days after source control = longer
- ✔ **SAPS** 2016, **ADAPT-Sepsis**: PCT-guided stopping ↓ duration
- ✔ **BLING III** 2024: continuous β-lactam (meta-analysis ↓ mortality)
- ✖ **MERINO** 2018: pip-taz worse than meropenem for ESBL bacteraemia

Key point: Shorter, guided, extended-infusion.

4. ARDS and ventilation

- ✔ **ARMA** 2000: 6 mL/kg ↓ mortality
- ✔ **PROSEVA** 2013: prone ≥ 16 h ↓ mortality
- ✖ **ART** 2017: aggressive recruitment ↑ mortality
- ✔ **ACURASYS** 2010 / = **ROSE** 2019: NMB
- = **EOLIA** 2018 (35 vs 46%), ✔ **CESAR** 2009: ECMO
- ✔ **FLORALI** 2015: HFNO
- ✔ **FACTT** 2006: conservative fluids ↑ ventilator-free days
- = **OSCILLATE/OSCAR**: HFOV no benefit (✖ OSCILLATE)

Key point: Low Vt, prone early, no aggressive recruitment.

5. Weaning and airway

- = **TracMan** 2013: early trach no benefit
- ✔ **Subirà** 2019: PSV 30 min > T-piece 2 h
- ✔ **HIGH-WEAN** 2019: NIV + HFNO after extubation in high risk
- ✔ **ABC** 2008: paired SAT + SBT ↓ mortality
- = **SETPOINT2** 2022: early trach in stroke

Key point: Daily SAT + SBT; trach for comfort, not survival.

6. Neuro

- ✖ **DECRA** 2011: early DC worse
- ✔/✖ **RESCUEicp** 2016: ↓ death, ↑ severe disability
- ✖ **Eurotherm3235** 2015; = **POLAR** 2018: hypothermia
- ✔ **CRASH-3** 2019: TXA in mild-moderate TBI; ✖ **CRASH** 2004: steroids
- = **ESETT** 2019: LEV = fosPHT = VPA
- ✔ **RAMPART** 2012: IM midazolam
- ✔ **TRAIN** 2024: liberal Hb in brain injury
- = **TTM2** 2021: 33 °C = normothermia after arrest

Key point: Lower ICP does not equal better outcome.

7. Renal, bleeding and others

- ✔ **BICAR-ICU** 2018: bicarbonate in pH ≤ 7.2 + AKI
- = **STARRT-AKI** 2020 / **AKIKI**: early RRT no benefit
- = **RENAL** / **ATN**: RRT intensity
- ✔ **Villanueva** 2013: Hb 70 in GI bleed
- ✖ **HALT-IT** 2020: TXA in GI bleed ↑ VTE
- ✔ **CRASH-2** 2010 / **WOMAN** 2017: TXA < 3 h
- ✔ **PROPPR** 2015: 1:1:1 ↓ exsanguination
- ✔ **TRICC** 1999: Hb 70 restrictive
- ✖ **NICE-SUGAR** 2009: tight glucose ↑ mortality

Key point: Restrictive transfusion and glucose 6–10.